



PSYCHOLOGICAL THERAPIES



**psych
scene**
RANZCP EXAM PREP



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PSYCHOLOGICAL THERAPIES

CBT

- Collaborative empiricism with guided discovery using Socratic questioning. - Aim to ask sequences of questions to establish a core dysfunctional assumption about self, world or future. Provide information that can help individual understand more about how he/she feels
- Identifying negative automatic thoughts and schemas using thought diaries
- Modify negative automatic thoughts, restructure core beliefs and schemas. Use thought challenges worksheet to challenge extreme and unhelpful thoughts
- Behavioural deficits – behavioural activation or pleasant event scheduling, graded task assignment, self-reward i.e. mastery and pleasure ratings- this is to identify the presence of the vicious circle of reduced activity
- Graded exposure with or without response loop tapes, event rehearsal (for nightmares and PTSD) and stress inoculation – **mainly for anxiety and panic**
- Problem solving
- Relaxation, re-breathing, stimulus control, visualisation, worry periods, and mindfulness meditation.
- Sleep hygiene
- Homework task – putting into practice what you have learnt
- Compliance therapy (see earlier)
- Relapse prevention strategy

CBT has a good evidence base for the treatment of depression, bulimia nervosa, Anxiety disorders (Panic disorder, phobia, GAD, social phobia, agoraphobia, PTSD, OCD, BDD), and Bipolar disorder in combination with medication, resistant delusions and hallucinations in schizophrenia and drug and alcohol misuse. Treatment of choice in first-episode psychosis

COMPONENTS OF INTERPERSONAL THERAPY

- Unresolved grief (any loss) – facilitate the mourning process
- Role disputes- helping client identify nature of the dispute, decide on a plan of action, begin to modify unsatisfying patterns, reassess expectations of the relationship or both
- Role transitions- difficulty coping with major transitions in life e.g. retirement, career move, moving, leaving home
- Interpersonal deficits- history of unsupportive and inadequate relationships

Evidence base in Depression, Bulimia nervosa, HIV associated depression.

Interpersonal and Social Rhythm Therapy (IPSRT), developed by Dr Ellen Frank and colleagues at the [University of Pittsburgh](#), is based on the idea that disruptions in daily routines and problems in interpersonal relationships can cause recurrence of the [manic](#) and [depressive](#) episodes that characterise [bipolar disorder](#)

DIALECTICAL BEHAVIOURAL THERAPY

- Developed by Marsha Linehan
- Based on the bio-social theory for Borderline Personality Disorder
- Combination of biological dysfunction in the emotional regulation system plus a socially invalidating environment leads to pervasive emotional dysregulation setting up a vicious circle

COMPONENTS OF DBT

Out-patient treatment delivered over a period of one year

1. *Individual sessions*

- Last for 45–60 minutes and occur weekly.
- Life-threatening behaviours
- Therapy-interfering behaviours
- Quality-of-life-interfering behaviours
- Attention to skills.

2. *Skills training group*

- Skills training is conducted in a weekly group, which typically lasts for 2 or 2½ hours.
- The style of the group is didactic.
- The skills training is organised around a manual that sets out the content of the programme in detail and gives advice about how it should be taught (Linehan, 1993b).

Four modules

- a. Emotional regulation
- b. Distress tolerance
- c. Interpersonal effectiveness
- d. Mindfulness

3. *Out-of-hours telephone contact*

- Brief, 5–10 minutes
- To help the patient avoid self-harm
- Planned contract between patient and therapist

4. *The consultation group*

- Team – the individual therapists and skills trainers – meet to review the programme and their practice.

COMPONENTS OF FAMILY THERAPY IN SCHIZOPHRENIA

- Family psychoeducation
- Building a therapeutic alliance with the family
- Address high EE
- Increase problem solving capacity of family
- Enhancing communication through circular questioning
- Boundary setting

- Promoting individual independence
- Allowing venting of anger and frustration
- Involving family in relapse prevention

ADDITIONAL COMPONENTS OF CBT IN SCHIZOPHRENIA

- Therapeutic alliance and empathy
- Challenging assumptions
- Reconceptualization of symptoms
- Mood modification
- Coping strategies and problem solving
- Compliance therapy
- Anxiety management

Techniques in Adherence therapy

- Reflective listening
- Regular summarising
- Inductive questioning
- Exploring ambivalence
- Use normalising rationales

MOTIVATIONAL INTERVIEWING

Principles of motivational interviewing

Developed by Miller and Rollnick and these principles are aimed at guiding the individual through the transtheoretical model of change

- **Express empathy**
- **Develop discrepancy**
- **Express empathy**
- **Support self-efficacy**

Evaluated in Project Match and UKATT study for alcohol dependence, and was found to be equal in benefit to AA, CBT (Marlatt and Gordon) and Social and Behaviour network therapy

Transtheoretical model of change

- Precontemplation
- Contemplation
- Determination
- Action
- Maintenance

Prochaska and DiClemente

Indications of Motivational interviewing in Psychiatry

- Drug and alcohol dependence
- Compliance therapy
- Bulimia

- Diabetes
- Mixed results in Smoking

BORDERLINE PERSONALITY DISORDER

Formulation Models

Childhood abuse and neglect are extremely common, with up to 87% of patients diagnosed with Borderline personality disorder having suffered childhood trauma of some sort (Perry & Herman 1993)

1. RISK

- Patients with Borderline Personality Disorder (BPD) commonly experience suicidal ideation (8–10% commit suicide)

2. Clarification of Diagnosis

- **Establish co morbidity** – Depression, Bipolar disorder, drug and alcohol misuse, eating disorders.
Disorder can only be established by collateral as the pervasive and enduring pattern of inner experience and distress with social and occupational difficulties need to be verified by family and friends.

3. Principles in management of BPD

Source: Fagin (2004)

- Careful assessments by experienced staff, focusing on the present crisis and need for containment
- Involvement of significant others, carers, relatives and other agencies in the assessment
- Early care plan with specified goals agreed and communicated to all staff and the patient, paying special attention to perceived or real inconsistencies. Anticipation of crises, especially about impulsive discharge, self-harm, drug use, sexual promiscuity or aggression, and establishment an agreed multidisciplinary response
- A focus on immediate needs, mostly of a practical nature
- Clear boundaries regarding tolerable behaviour, including aggression, suicidal gestures, use of illicit substances or alcohol and absconding
- Effective use of in-patient groups
- Treatment of psychiatric symptoms with medication when necessary
- Staff support groups and supervision looking at counter-transference reactions, particularly of junior staff, who may become over involved
- Early discharge arrangements when crisis has been overcome
- Readiness to discharge if goals are not met
- Referral to community or specialist services on discharge, with close and careful handovers through the discharge plans
- Short duration of admissions
- Consideration of co-morbidity of a personality disorder if treatment for an Axis I disorder does not meet with the predictable response

Maintain flexibility	Staff should be aware of their potential lack of responsiveness as a defence against the extreme feelings that these patients engender. Patients are likely to respond very negatively to stereotypical or rigid stances, and staff need to be spontaneous as far as possible yet keep professional boundaries (not an easy task).
Establish conditions that keep the patient safe	Set boundaries regarding the need for hospitalisation, suicidal behaviour, use of drugs and alcohol, and inappropriate crossing of professional boundaries
Tolerate intense anger, aggression and hate	Remember that most patients are trying to re-establish their relationship with the rejecting parent by creating a sadomasochistic attachment to staff. Attempts at avoiding such projections, either by defensive countermeasures, trying to prove that the staff is in fact good, or by angry retorts and rejections are likely to lead to disengagement.
Promote reflection	"What triggered that extreme reaction?" "How do you think I felt when you attacked me in that way?" "What do you think are the consequences of cutting your wrists?"
Set necessary limits	These need to be clarified if behaviours threaten the safety of the patient, family, staff or the therapeutic relationship.
Establish and maintain the therapeutic alliance	Regularly revisit the aims and goals of the therapeutic contract, especially when either the patient or staff have lost their way.
Avoid splitting between psychotherapy and pharmacotherapy	Patients' responses to prescribed medication must form part of the therapeutic interactions and be discussed openly if medication is resisted, sabotaged or abused. Patients may need reminding that the aims of medication are modest: to alleviate the most distressing symptoms and allow time and opportunity to reflect and share feelings.
Avoid or understand splitting between members of staff, either in hospital or in the community	Recognise that patients may display completely different attitudes, both loving and attacking, within short periods, which can be quite perplexing to staff. Patients are projecting these aspects of themselves onto others, as a way of trying to control these fragmented parts of themselves.
Monitor counter-transference feelings	Enable staff to share embarrassing or difficult feelings prompted by their involvement with these patients

Psychological Treatments for BPD (Tyrer 2004; APA guidelines)

Psychodynamic therapy	To increase reflective capacity and emotional and interpersonal understanding
Cognitive behavioural therapy	To alter dysfunctional core beliefs
Dialectical behaviour therapy	Initially, to reduce self-harm; eventually, to achieve transcendence
Therapeutic community	To effect attitudinal and behavioural change
Cognitive analytic therapy	To achieve greater self-understanding
Behaviour therapy	To improve maladaptive behaviour
Nidotherapy	To achieve better environmental adjustment, thus minimising impact of disorder (Tyrer 2002a)

Drug treatment for personality disorders (Soloff 1998)

COGNITIVE/PERCEPTUAL Suspiciousness, paranoid ideation, ideas of reference, odd communication, muddled thinking, magical thinking, episodic distortions of reality, derealisation, depersonalisation, illusions, stress-induced hallucinations	Antipsychotics
AFFECTIVE DYSREGULATION Lability of mood, 'rejection sensitivity', mood crashes, inappropriate intense anger, temper outbursts, chronic emptiness, dysphoria, loneliness, anhedonia, social anxiety and avoidance	SSRIs, MAOIs
IMPULSIVE-BEHAVIOURAL DYSCONTROL Sensation-seeking, risky or reckless behaviour, no reflective delay, low frustration tolerance, impulsive aggression, recurrent assaultiveness, threats, property destruction, impulsive binges (drugs, alcohol, food, sex, spending), recurrent suicidal threats and behaviour, some self-mutilation	Mood stabilisers, SSRIs

POST-TRAUMATIC STRESS DISORDER (PTSD)

AIMS OF TREATMENT

- Reduce the severity of Acute Stress Disorder or PTSD symptoms
- Prevent or reduce trauma-related co-morbid conditions
- Improve adaptive functioning and restore or promote normal developmental progression
- Protect against relapse
- Integrate the danger experienced as a result of the traumatic situation(s) into a constructive schema of risk, safety, prevention, and protection

Treatment of ASD or PTSD symptoms includes three broad categories of intervention:

Pharmacological treatment

- SSRIs are recommended as first-line medication treatment for PTSD.
- Other antidepressants may also be beneficial.

Advantages of SSRIs:

- Ameliorate all three PTSD symptom clusters (i.e., reexperiencing, avoidance/numbing and hyperarousal),
- Effective treatments for psychiatric disorders that are frequently co-morbid with PTSD (e.g., depression, panic disorder, social phobia, and OCD),
- Reduce clinical symptoms (such as suicidal, impulsive, and aggressive behaviours) that often complicate management of PTSD, and
- Relatively few side effects.

Psychotherapeutic interventions

Cognitive behavioural therapy

- Effective treatment for core symptoms of PTSD
- Cognitive behaviour therapy in ASD or PTSD targets the distorted threat appraisal process (in some instances through repeated exposure and in others through techniques focusing on information processing without repeated exposure) in an effort to desensitise the patient to trauma-related triggers.

EMDR

- EMDR is a form of psychotherapy that includes an exposure-based therapy (with multiple brief, interrupted exposures to traumatic material), eye movement, and recall and verbalisation of traumatic memories of an event or events. It therefore combines multiple theoretical perspectives and techniques, including CBT.

Stress inoculation, imagery rehearsal, and prolonged exposure techniques

Education and supportive measures

Psychological debriefing is not effective

RANZCP GUIDELINES RECOMMENDATIONS

Psychological therapy

- Adults with PTSD should be provided with trauma focused interventions (trauma focused CBT or EMDR in addition to *in vivo* exposure).
- Non-trauma-focused interventions such as supportive counselling and relaxation should not be provided to adults with PTSD in preference to trauma-focused interventions.
- Where symptoms have not responded to a range of trauma-focused interventions, evidence-based non-trauma-focused interventions (such as stress management) and/or pharmacotherapy should be considered.
- Sessions that involve imaginal exposure generally require 90 minutes.
- Following assessment, diagnosis and treatment planning, 8 to 12 sessions of trauma-focused treatment is usually sufficient.

Pharmacological interventions

- Drug treatments for PTSD should not be used as a routine first-line treatment for adults, either by general medical practitioners or by specialist mental health professionals, in preference to trauma-focused psychological therapy.
- Where medication is considered for the treatment of PTSD in adults, SSRI antidepressants should be the first choice for both general practitioners and mental health specialists.
- Other new generation antidepressants (notably Mirtazapine) and the older tricyclic antidepressants should be considered as a second-line option. Phenelzine should be considered for use by mental health specialists for patients with treatment-resistant symptoms
- When an adult sufferer with PTSD has responded to drug treatment, it should be continued for at least 12 months before gradual withdrawal.

CHILD AND ADOLESCENT PSYCHIATRY

Autism Spectrum Disorder (DSM-V):

1. Persistent deficits in social communication and social interaction across multiple contexts, as manifested by
 - a) Deficits in social-emotional reciprocity
 - b) Deficits in nonverbal communicative behaviours used for social interaction
 - c) Deficits in developing, maintaining, and understanding relationships
2. Restricted, repetitive patterns of behaviour, interests, or activities manifested by
 - a. Stereotyped or repetitive motor movements, use of objects or speech
 - b. Insistence on sameness, inflexible adherence to routines
 - c. Highly restricted, fixated interests that are abnormal in intensity or focus
 - d. Hyper- or hypo reactivity to sensory input or unusual interest in sensory aspects of the environment

IMPORTANT!

Aspergers has features of autism without the language abnormalities

Autism is associated with fragile X syndrome, rubella, tuberous sclerosis, phenylketonuria and Rett's syndrome

In DSM 5 the disorder is collapsed under Autism Spectrum Disorder: The two criteria include

- Persistent deficits in social communication and social interaction across multiple contexts
- Restricted, Repetitive patterns of behaviour, interests or activities.

SEPARATION ANXIETY DISORDER

Developmentally inappropriate and excessive anxiety concerning separation from home or from those to whom the individual is attached, as evidenced by three (or more) of the following:

- recurrent excessive distress when separation from home or major attachment figures occurs or is anticipated
- persistent and excessive worry about losing, or about possible harm befalling, major attachment figures
- persistent and excessive worry that an untoward event will lead to separation from a major attachment figure (e.g. getting lost or being kidnapped)
- persistent reluctance or refusal to go to school or elsewhere because of fear of separation
- persistently and excessively fearful or reluctant to be alone or without major attachment figures at home or without significant adults in other settings
- persistent reluctance or refusal to go to sleep without being near a major attachment figure or to sleep away from home
- repeated nightmares involving the theme of separation
- repeated complaints of physical symptoms (such as headaches, stomach aches, nausea, or vomiting) when separation from major attachment figures occurs or is anticipated
- The duration of the disturbance is at least 4 weeks.
- The disturbance causes clinically significant distress or impairment in social, academic (occupational), or other important areas of functioning

TREATMENT

- Return to school is priority to avoid disruption to education
- This is done in liaison with parents and school psychologists
- Specific behavioural programme based on graded desensitisation
- Treat the psychiatric condition
- Use specific techniques from cognitive behavioural therapy for anxiety and phobias
- Chronic problem may require assessment in a psychiatric unit
- Home education may be required
- Family therapy to address any family issues

General template for any child and adolescent psychiatry question that involves assessment:

Assessments in Child and Adolescent Psychiatry are done in three stages; assessment of child alone, child and family, and parents alone.

I. ASSESSMENT:

- History about onset, duration, triggers, ameliorating factors, effect on family and treatment sought so far
- Assessment of other physical health problems
- Interests, activities and hobbies
- Relationship with parents, siblings
- Relationship with other friends
- Mood
- Antisocial behaviour
- Schooling – attendance, relationships, academics

II. FAMILY STRUCTURE AND RELATIONSHIPS:

- Parent-child relationship
- Parent-parent relationship
- Psychosocial issues affecting family – relationship issues, finances

III. PERSONAL HISTORY, I.E. DEVELOPMENTAL HISTORY

- Pregnancy and delivery
- Early mother-child relationship: post-partum depression
- Temperamental characteristics: easy or difficult
- Developmental milestones
- Past illnesses and injuries, episodes of hospitalisation
- Separations greater than one week

IV. MSE

- Appearance
- Activity level
- Mood
- Reaction to and relationship with the doctor
- Behaviour with parents
- Habits and mannerisms
- Presence of delusions, hallucinations and thought disorder

V. OBSERVE FAMILY RELATIONSHIPS

- Patterns of interaction
- Clarity of boundaries between parents and child
- Communication
- Emotional atmosphere of family

VI. PHYSICAL EXAMINATION

- Screening neurological examination

- Especially observe for abnormal movements, tics, mannerisms

VII. THE MAIN DIFFERENTIALS FOR SCHOOL REFUSAL INCLUDE:

- Separation anxiety
- School phobia
- Other specific phobia
- Bullying at school
- Other broader psychiatric issue, e.g. depression or low self esteem

IMPORTANT POINT: In psychiatric disorders with children, CBT is first line. Fluoxetine is recommended for anxiety and depression in children and OCD. There is no conclusive evidence about increased suicidal risk, but parents must be informed to detect any treatment-emergent suicidal thoughts or activation, agitation (RANZCP).

CRITERIA FOR CONDUCT DISORDER

- Aggression to people and animals
 - Destruction of property
 - Deceitfulness or theft
 - Serious violation of rules
- High correlation with antisocial personality disorder

CRITERIA FOR ADHD

- Either Inattention or hyperactivity-impulsivity

Inattention

Six (or more) of the following symptoms of inattention have persisted for at least 6 months to a degree that is maladaptive and inconsistent with developmental level:

- often fails to give close attention to details or makes careless mistakes in schoolwork, work, or other activities
 - often has difficulty sustaining attention in tasks or play activities
 - often does not seem to listen when spoken to directly
 - often does not follow through on instructions and fails to finish schoolwork, chores, or duties in the workplace (not due to oppositional behaviour or failure to understand instructions)
 - often has difficulty organising tasks and activities
 - often avoids, dislikes, or is reluctant to engage in tasks that require sustained mental effort (such as schoolwork or homework)
 - often loses things necessary for tasks or activities (e.g., toys, school assignments, pencils, books, or tools)
 - is often easily distracted by extraneous stimuli
 - is often forgetful in daily activities
- **Hyperactivity-impulsivity:** six (or more) of the following symptoms of hyperactivity-impulsivity have persisted for at least 6 months to a degree that is maladaptive and inconsistent with developmental level:

- ***Hyperactivity***
 - often fidgets with hands or feet or squirms in seat
 - often leaves seat in classroom or in other situations in which remaining seated is expected
 - often runs about or climbs excessively in situations in which it is inappropriate (in adolescents or adults, may be limited to subjective feelings of restlessness)
 - often has difficulty playing or engaging in leisure activities quietly
 - is often 'on the go' or often acts as if 'driven by a motor'
 - often talks excessively
- ***Impulsivity***
 - often blurts out answers before questions have been completed
 - often has difficulty awaiting turn
 - often interrupts or intrudes on others (e.g., butts into conversations or games)
- Some hyperactive-impulsive or inattentive symptoms that caused impairment were present before 7 years of age.
- Some impairment from the symptoms is present in two or more settings (e.g. at school [or work] and at home).
- There must be clear evidence of clinically significant impairment in social, academic, or occupational functioning.

MANAGEMENT OF ADHD

Assessment as above in school refusal

Important additional points include (RANZCP):

- A comprehensive multi-modal assessment including structured observations of child in a variety of situations over time
- Exclude developmentally appropriate levels of inattention and other disorders like autism
- Environmental and family assessment
- Psychometric assessment/language and cognition
- Neurological examination

Management

- Involve family, school and social network
- Monitoring of learning, socioeconomic development and behaviour
- Specialist child psychiatrist referral
- Involve paediatrician and neurologist as appropriate
- The recent MTA study (NIMH 1999) advocates a multimodal approach involving drug treatment, a psycho-educational and parenting skills programme, and individual and group work with the child
- Stimulants (Methylphenidate or dexamphetamine): Monitor BP, pulse, weight, sleep and cardiac status. Atomoxetine can be considered if side effects like tics occur with stimulants.

FEATURES AND MANAGEMENT OF ENURESIS

- Involuntary passage of urine in the absence of physical abnormality after the age of 5 years
- Divided in diurnal or nocturnal enuresis
- 25% have signs of psychiatric disturbance
- 10% of 5-year olds, 5% of 10-year olds, and 1% of 18-year olds

Aetiological factors associated with enuresis:

- Positive family history (70%)
- Lower intelligence
- Psychiatric disturbance, e.g. anxiety
- Stressful life events
- Large family size
- Social disadvantage

MANAGEMENT:

- Assessment of psychiatric disturbance according to principles in previous questions
- Neurological examination and full physical examination
- abdominal examination for enlarged bladder or kidneys or faecal masses indicating encopresis
- genital examination for hypospadias, epispadias, meatal stenosis, ectopic ureter, or labial adhesions; and rectal examination of perianal and perineal sensation and rectal sphincter tone and for detection of perianal excoriation or vulvovaginitis.
- Focused neurologic evaluation should include examination of gait, muscle tone, strength, and perineal sensation as well as a search for spinal pathologic process, deformity, or sacral dimple or hair tuft suggesting underlying spinal dysraphism.
- Urinalysis and urine culture hormone level for hyperthyroidism.
- Specific imaging studies may be indicated, such as renal and bladder ultrasonography and voiding cystourethrography for suspected structural abnormality, significant daytime wetting, or recurrent urinary infections to detect vesicoureteral reflux.
- Magnetic resonance imaging of the lumbosacral spine should help diagnose suspected spinal dysraphism or underlying cause for abnormal neurologic examination findings. To evaluate bladder dysfunction, urodynamic studies may include measurement of residual urine and cystometry.
- Treatment should be postponed until the child is willing and able to comply with the treatment programme
- Treatment is not necessary if the child is not distressed about the enuresis, but parents should be reassured about their child's physical and emotional health and counselled regarding eliminating guilt, shame and punishment.
- Educate parents
- Explain that in most cases the child will grow out of it and favourable prognosis
- Star chart – accurate recording of enuresis plus positive reinforcement for dry nights
- Toilet training – based on chaining and positive reinforcement
- Enuresis alarm

- Combination of star chart and alarm
- Desmopressin and imipramine

CLINICAL FEATURES AND MANAGEMENT OF ENCOPRESIS

- Inappropriate passage of formed faeces in the absence of physical pathology after 4 years of age
- Divided into retentive and non-retentive (anger/aggression/hostility)
- Non-retentive is divided into continuous (inadequate training) and discontinuous (anxious/phobic)

MANAGEMENT

- Assessment as above
- Physical and neurological examination
- Investigation of the bowels
- Exclude Hirschsprung's disease
- The main aims of treatment are:
Promote normal bowel habit
Promote parent-child relationship
- Judicious use of smooth muscle stimulants, stool softeners and bulk agents
- Parental and child education about the importance of fibre in diet
- Star charts
- Behavioural psychotherapy

DSM 5 CRITERIA FOR TOURETTE'S SYNDROME

- Both multiple motor tics and one or more vocal tics must be present at the same time, although not necessarily concurrently
- The tics may wax and wane in frequency but have persisted for more than 1 year since first tic onset
- The age at onset must be less than 18 years;
- The disturbance must not be due to the direct physiological effects of a substance (e.g. stimulants) or a general medical condition (e.g. Huntington's disease or postviral encephalitis).

Medications effective in Tourette's Syndrome

- **Haloperidol**
- **Pimozide (not available due to prolonged QT interval)**
- Clonidine
- SSRIs for comorbid OCD
- Atomoxetine for ADHD as does not worsen tics

PRINCIPLES OF ASSESSMENT AND MANAGEMENT OF ADULT ADHD

Many children with ADHD (65–85%) will continue to meet ADHD criteria into adulthood (CNS Spect 2007).

While the DSM-IV criteria for ADHD may be applied to adults, these dimensions tend to reflect presentations in children. The subtlety of ADHD symptoms among adults has led to several modifications of existing criteria. Rather than requiring six DSM-IV symptoms of inattention or hyperactivity, some investigators propose requiring only five such behaviours for older patients. In addition, the symptoms take different forms in adults.

Inattentive traits are more likely than impulsive-hyperactive traits to be a chief complaint in adults, as impulsive-hyperactive symptoms tend to decline as the person matures (Biol Psych 2000).

Principles in assessment

- Obtain a developmental history. Attempt to corroborate information with other sources, such as parents, spouse and school report cards. Symptoms should be consistently present since early childhood.
- Inquire about impact of core ADHD symptoms on current occupational, school and relationship functioning.
- Assess attention, concentration, distractibility and short-term memory by having patient perform screening tasks in the office setting.
- **The differential diagnosis of ADHD should rule out other forms of adult encephalopathy that affect cognitive performance.** These can include neurologic dysfunction attributable to heavy metal or other poisoning, metabolic disturbance including B12 or thyroid conditions, sleep disorders such as sleep apnea, lingering infectious processes such as mononucleosis, or less reversible conditions such as multiple sclerosis or dementia processes. Furthermore, the differential diagnosis of ADHD should include physical and mental states that could render sufficient distraction to cause poor performance in major life areas. Physical or emotional pain, for example, will often produce poor performance. (J Clin Psych 2009)
- Approximately 70–75% of adults with ADHD will have at least one co-existing psychiatric diagnosis and up to 90% of adults with ADHD will have a clinically significant lifetime co-morbid disorder. The most common psychiatric comorbidities include major depressive disorder, social phobia, bipolar disorder, anxiety and alcohol dependence (Annu Rev Med 2002).
- Assess for the presence of other psychiatric disorders and substance
- Neurological examination and laboratory tests

Screening tools for ADHD

- Copeland Symptom Checklist for Adult ADHD
- Wender Utah Rating Scale

- Brown Adult Attention Deficit Disorder Scale
- Adult ADHD Self-report Scale (ASRS) The ASRS screener is a 6-item subset of the 18-item self-report form, with a question for each of the 18 symptoms of inattention or impulsivity-hyperactivity. This is the most widely used and is accepted by the WHO.

Diagnostic tools

- Adult ADHD Clinical Diagnostic Scale (ACDS)
- Conners' Adult ADHD Diagnostic Interview for DSM-IV (CAADID)

MANAGEMENT

Pharmacological interventions

A meta-analysis has suggested that, in general, stimulants may be more effective than nonstimulants for the management of ADHD. These results have been extrapolated from studies in children and adolescents (Faraone 2003, Pliszka, 2007)

Atomoxetine carries warnings about very rare liver failure, and the possibility of suicidal ideation. Patients should always be warned that unexpected thoughts and feelings as well as physical discomforts are possible side effects of medications.

In a patient with no comorbidity, stimulants are typically the first choice because of their more robust efficacy for ADHD management. Atomoxetine and other nonstimulants should be tried for patients who have not tolerated or benefited sufficiently from trials with methylphenidate and amphetamine.

Long-acting stimulant therapies are recommended as first-line treatments over short-acting stimulants for a few reasons. They are more difficult to abuse and have lower rates of misuse. Long-acting stimulants typically cover the most active part of the day in one dose, rather than requiring the added organisational task of dosing several times a day.

Anticipate and manage side effects

The catecholaminergic agents that treat ADHD have similar side effects. Both stimulants and nonstimulants commonly produce some form of peripheral side effects. The most common and potentially concerning of these is increased heart rate or blood pressure. Blood pressure and heart rate should be monitored at baseline and at subsequent follow-up visits.

Patients should be warned that, although unusual, any negative changes in mood, thoughts or personality should be reported, and they should be reassured that these changes will resolve with termination of treatment. Other physical effects such as dry mouth, loss of appetite, gastrointestinal upset, or muscle tension occur in difficult-to-predict patterns.

Pharmacological Interventions include methylphenidate, dextroamphetamines, desipramine, Imipramine, nortriptyline and bupropion. Clonidine can also be used though its benefit in adults is less clear.

Be aware of substance misuse and stimulant diversion.

SELF-MANAGEMENT STRATEGIES

Psychotherapy

- Preliminary evidence suggests that structured CBT and meta cognitive therapy may improve patients' compensatory strategies (Behav Res Ther. 2005, Am J Psych 2010).